

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Trimethoprim oral suspension 50mg/5ml and Cefalexin 125mg/5ml oral suspension for the treatment of Paediatric Urinary Tract Infection (UTI)

Summary of NICE / NICE CKS Guidelines for Paediatric Urinary Tract Infection (UTI)

NICE NG224: Urinary Tract Infection in Children and Young People

Paediatric UTI management is age-specific and depends on clinical presentation and imaging findings. Early diagnosis and appropriate antibiotic therapy are essential to prevent complications including renal scarring.

Age and Clinical Presentation

- Children aged 3 months to 16 years with suspected or confirmed UTI should be assessed for lower UTI (uncomplicated cystitis) or upper UTI (pyelonephritis).
- Lower UTI presents with dysuria, frequency, urgency, and suprapubic discomfort.
- Upper UTI presents with fever, flank pain, malaise, and systemic symptoms.

Imaging Criteria

- First episode of uncomplicated cystitis: no imaging required.
- First episode of pyelonephritis or recurrent UTI: ultrasound within 4 weeks recommended.
- Consider DMSA scan in specific clinical circumstances as per NICE guidance.

Antibiotic Selection

- First-line oral antibiotics for uncomplicated lower UTI: trimethoprim or cefalexin.
- For pyelonephritis or severe disease: consider parenteral therapy or second-line agents.
- Duration: 3 days for uncomplicated lower UTI; 7-10 days for upper UTI.

Patient Group Direction

for the supply/administration of

Trimethoprim oral suspension 50mg/5ml

for the treatment of

Paediatric Urinary Tract Infection (UTI)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC

Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of uncomplicated and complicated urinary tract infections in children aged 3 months to 16 years
Inclusion criteria	Children aged 3 months to 16 years Confirmed or suspected uncomplicated UTI (lower) or upper UTI (pyelonephritis) Parental/guardian consent obtained Able to take oral medication Baseline renal function normal (if available, or clinically assessed)
Exclusion criteria	Age less than 3 months (refer to paediatric specialist) Known hypersensitivity to trimethoprim or sulfonamides History of blood dyscrasias or megaloblastic anaemia Suspected folate deficiency Severe renal impairment (eGFR <30 mL/min/1.73m²) Pregnant or breastfeeding (applies to adolescent girls) Current use of other medications with significant interactions
Cautions	Monitor for symptoms of bone marrow suppression (bruising, sore throat, unusual tiredness) Adequate hydration should be maintained throughout treatment In children with recurrent UTI, consider underlying anatomical or functional abnormalities Use with caution in children with G6PD deficiency Folate supplementation may be considered for prolonged therapy

Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.
---	--

Details of the medicine

Name, form and strength of medicine	Trimethoprim oral suspension 50mg/5ml
Legal category	POM
Storage	Store at room temperature, protected from light. Keep in a tightly closed container.
Route/method of administration	Oral
Dose and frequency	4 mg/kg twice daily for 3 days (for uncomplicated lower UTI) 4 mg/kg twice daily for 7-10 days (for upper UTI/pyelonephritis) Doses should be calculated based on actual body weight
Quantity to be administered and/or supplied	Appropriate volume for age and weight to complete prescribed course
Maximum or minimum treatment period	3 days for lower UTI; 7-10 days for upper UTI
Adverse effects	Gastrointestinal upset (nausea, vomiting, diarrhoea) Rash (including photosensitivity) Blood dyscrasias (rare but serious: thrombocytopenia, agranulocytosis) Hepatotoxicity (rare) Stevens-Johnson syndrome (rare)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD

	Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: - Adults aged 18 years and over - keep records for 8 years - Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
--	---

Patient information

Written information to be given to patient or carer	Provide the patient information leaflet (PIL) supplied with the medication. Ensure parents/guardians understand the importance of completing the full course of antibiotics and adequate hydration.
Follow-up advice to be given to patient or carer	Complete the full course of antibiotics as prescribed, even if symptoms improve. Maintain adequate fluid intake (water, dilute juice) to promote normal urine flow. Use paracetamol or ibuprofen (at appropriate doses) for fever management as needed. Seek urgent medical attention if: fever worsens or persists beyond 48 hours of antibiotic start, vomiting prevents oral medication intake, signs of acute illness develop, or child becomes unwell. Return to GP for review if symptoms do not resolve within 48 hours of starting antibiotics. Contact GP or health visitor if child develops rash or signs of allergic reaction. For children with recurrent UTI, ensure follow-up with GP to arrange imaging and investigation as per NICE NG224.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: NICE NG224 Urinary Tract Infection in Children and Young People
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
------	---------------	-----------	------

Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply/administration of
Cefalexin 125mg/5ml oral suspension
for the treatment of
Paediatric Urinary Tract Infection (UTI)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of urinary tract infections in children aged 3 months to 16 years; alternative to trimethoprim or in cases of trimethoprim resistance or intolerance
Inclusion criteria	Children aged 3 months to 16 years Uncomplicated or complicated UTI Parental/guardian consent obtained Able to take oral medication No history of immediate hypersensitivity to cephalosporins
Exclusion criteria	Known cephalosporin allergy (especially immediate/IgE-mediated reactions)

	Severe penicillin allergy with cross-reactivity risk to cephalosporins Severe renal impairment (eGFR <30 mL/min/1.73m ²)
Cautions	History of penicillin allergy: risk of cross-reactivity approximately 1-3%; use with caution if penicillin allergy was non-severe Monitor for signs of allergic reaction during early treatment Use with caution in children with renal impairment (dose adjustment may be needed) Assess for previous adverse reactions to cephalosporins
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Cefalexin 125mg/5ml oral suspension
Legal category	POM
Storage	Store at room temperature. After reconstitution with water, refrigerate at 2-8°C and use within 10 days.
Route/method of administration	Oral
Dose and frequency	12.5 mg/kg twice daily for 3-7 days (standard therapy) 25 mg/kg twice daily for 3-7 days (for severe infections) Doses should be calculated based on actual body weight
Quantity to be administered and/or supplied	Appropriate volume for age and weight to complete prescribed course
Maximum or minimum treatment period	3-7 days
Adverse effects	Gastrointestinal upset (nausea, vomiting, diarrhoea, abdominal pain) Rash and urticaria Allergic reactions (uncommon, including anaphylaxis in severe cases) Photosensitivity (rare) Pseudomembranous colitis (rare, associated with prolonged use)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information:

	• that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
--	--

Patient information

Written information to be given to patient or carer	Provide the patient information leaflet (PIL) supplied with the medication. Ensure parents/guardians understand the importance of completing the full course of antibiotics and adequate hydration.
Follow-up advice to be given to patient or carer	Complete the full course of antibiotics as prescribed, even if symptoms improve. Maintain adequate fluid intake (water, dilute juice) to promote normal urine flow. Use paracetamol or ibuprofen (at appropriate doses) for fever management as needed. Seek urgent medical attention if: fever worsens or persists beyond 48 hours of antibiotic start, vomiting prevents oral medication intake, signs of acute illness develop, or child becomes unwell. Return to GP for review if symptoms do not resolve within 48 hours of starting antibiotics. Contact GP or health visitor if child develops rash or signs of allergic reaction. For children with recurrent UTI, ensure follow-up with GP to arrange imaging and investigation as per NICE NG224.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017

<https://www.nice.org.uk/guidance/mpg2/resources>

- NICE CKS Guidance: NICE NG224 Urinary Tract Infection in Children and Young People
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025